

1. Statement of Need

The product and its impact. The gap between older Americans who need help and the care available to them is growing. An aging population is increasing demand while families continue to face a fragmented system for finding providers, securing financial assistance, and establishing care. When care is not established, the consequences can compound into a vicious cycle of unmet need, hospitalization, failed care establishment, and premature institutionalization (Figure 1). Among community-dwelling Medicaid HCBS users, unmet service needs have been associated with substantially greater emergency-department use (52% vs. 34%) and hospital or rehabilitation stays (36% vs. 24%).¹ Olera developed CareNavigator through NIA Phase I–IIB (Impact Scores 20 and 25) to intervene earlier in this cycle by helping families identify appropriate care, find aid to pay for it, and connect with providers who can deliver it. Its central outcome is simple and measurable: does a recognized need reach established care?

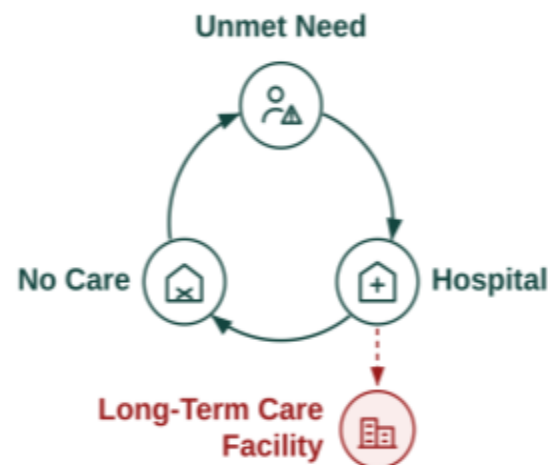


Figure 1. The vicious cycle of unmet need.

The remaining opportunity is to make the full care-establishment pathway computable, executable, and measurable from recognized need through confirmed aid or care (Figure 2). Prior work substantially developed the upstream functions needed to assess household needs and means, identify likely benefits and aid, and match appropriate services and providers. The CRP adds three integrated capabilities: (1) a longitudinal Care Establishment Model that represents needs, means, funding, service, execution, capacity, and outcome as structured case state; (2) bounded AI agent workflows that execute and learn from the administrative work required to move that state toward established care; and (3) data-directed Caregiver Staffing that adds new workforce capacity when insufficient labor prevents care establishment.

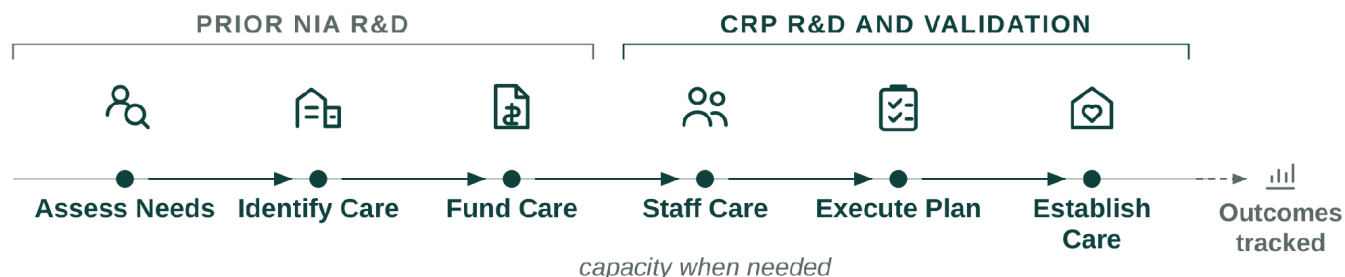


Figure 2. Care establishment requires a coordinated pathway from assessing need through confirming care.

Olera's Valley of Death (Figure 3). CareNavigator is deployed nationally, draws 15,500+ visitors per month through organic search at near-zero acquisition cost, and has demonstrated usability and technology acceptance in peer-reviewed studies.^{2,3} Caregiver Staffing has also been tested in prior pilots, where providers hired workers sourced through Olera and demonstrated willingness to pay for the service.⁴ Family demand, CareNavigator usability, and basic provider demand for Caregiver Staffing are therefore substantially de-risked. Five remaining risks must now be retired in sequence:

1. **Technical risk.** Can CareNavigator execute and track care establishment?
2. **Real-world validation risk.** Does the complete pathway work in practice?
3. **Evidence risk.** Does establishing care produce outcomes and economic value institutional buyers care about?
4. **Commercial risk.** Can the value created support durable revenue?
5. **Financing risk.** Is Olera investable when CRP ends?

TODAY

Demonstrated demand

15,500+ monthly visitors
CareNavigator usability validated
Staffing willingness to pay shown

AT CRP COMPLETION

Commercial sustainability

Repeatable provider revenue
Institutional evidence package
Positioned for private investment

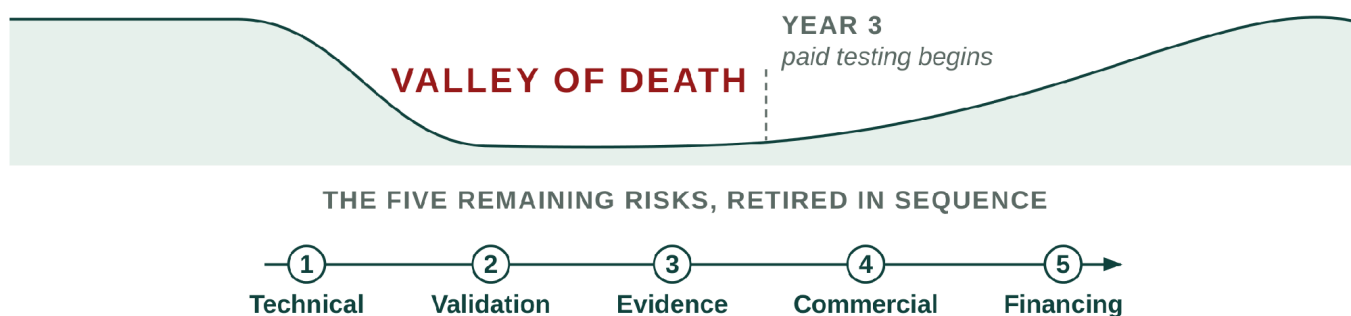


Figure 3. CRP bridges the five remaining risks between demonstrated demand and commercial sustainability.

Why government funding is the right instrument at this stage. Crossing this gap requires later-stage R&D and evidence generation before CareNavigator's largest commercial pathway can be demonstrated. Private investors must underwrite the risk of completing and deploying the system before its institutional value has been established, while institutional buyers need real-world evidence before they can confidently value and purchase the product. Non-dilutive CRP funding can break this cycle by financing the work needed to retire these risks.

The alternative is not simply to raise prices or sell the same product differently. Charging families would create the greatest barrier for households already struggling to afford eldercare. Charging providers for referrals would introduce steering incentives and limit participation by some federally reimbursed providers. Caregiver Staffing can generate nearer-term provider revenue without those tradeoffs, but staffing alone addresses only the workforce barrier; it does not help families navigate, fund, and execute the rest of the care-establishment pathway.

Our investor advisors agree that longitudinal outcomes demonstrating CareNavigator's value to institutional buyers, together with a repeatable provider-revenue model, would materially improve Olera's investability.

How CRP funding advances Olera to full commercialization. Three sequential aims retire the remaining risks. Aim 1 engineers and technically verifies the integrated care-establishment infrastructure and establishes its safe automation envelope. Aim 2 tests that system across eight free validation markets, measuring verified care establishment, agent execution, stakeholder value, workforce throughput, acquisition, and operating cost. Aim 3 then enters eight new paid markets to test conversion, retention, repeatable market activation, and live unit economics while producing the institutional evidence package and post-CRP payer proof-of-concept specification.

2. Value of the CRP Project, Expected Outcomes, and Impact

The product to be commercialized. CareNavigator is Olera's family-facing eldercare navigation platform. It combines a national, expert-curated resource database with AI-supported execution workflows and longitudinal outcomes tracking to help families move from recognized need to established care. Families use CareNavigator at no cost. The platform assesses needs, identifies appropriate care and financial aid, helps execute the administrative and follow-up work required to obtain them, confirms whether care was established, and records where the pathway succeeds or fails (Figure 4).

Caregiver Staffing is a complementary provider-facing product and capacity mechanism. When an otherwise appropriate care plan cannot be delivered because a provider lacks workers, Olera recruits new caregivers into the workforce and connects them with licensed providers, which retain responsibility for interviewing, hiring, training, credentialing, supervision, and care delivery.

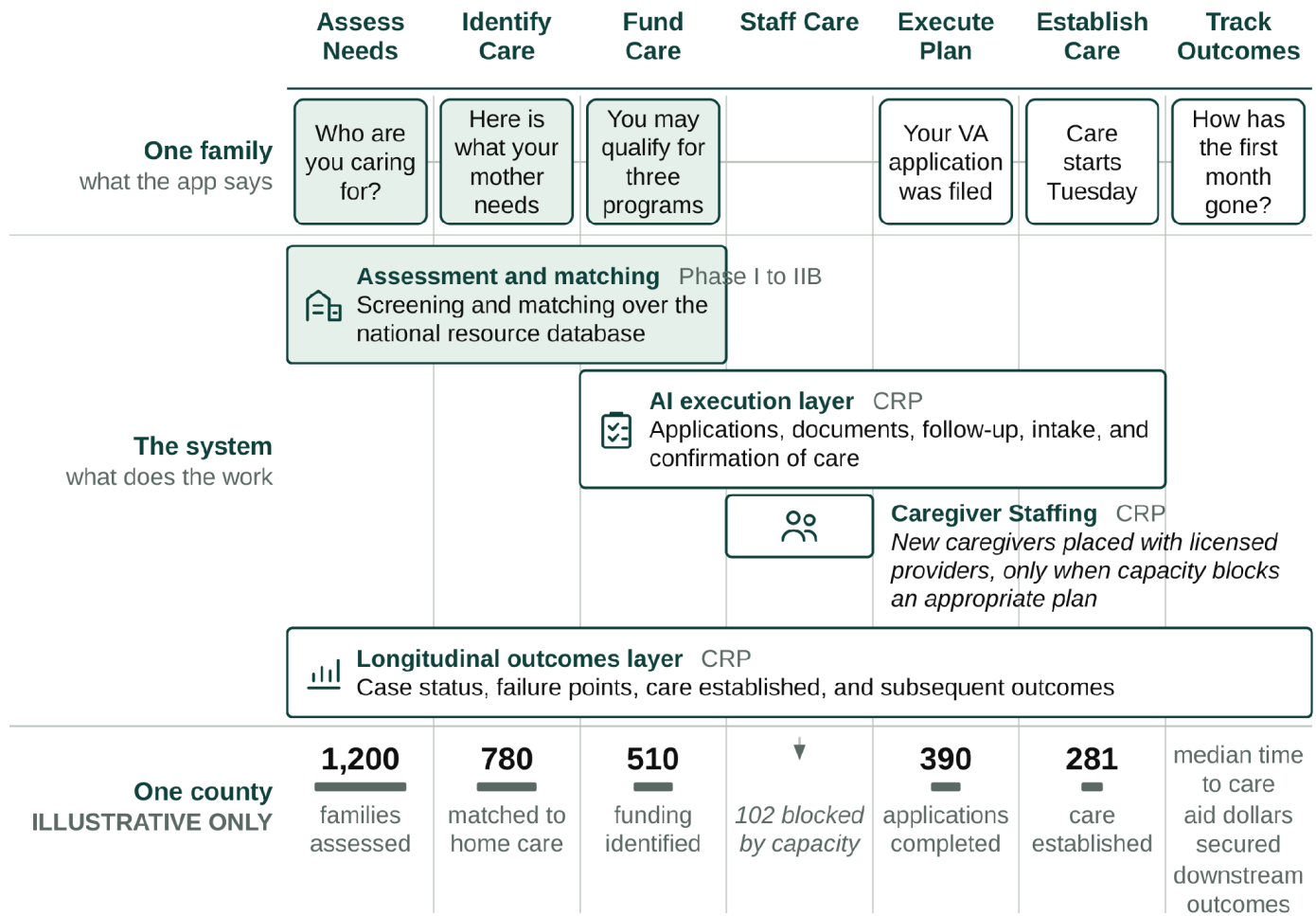


Figure 4. What a family sees, what the system does, and what accumulates across a county. Shaded elements exist today.

Foundation from prior SBIR R&D. Olera's NIA Phase I and Phase IIB awards (Impact Scores 20 and 25) established the foundation for this commercialization effort: a nationally deployed first-generation CareNavigator; an expert-curated database containing more than 72,000 eldercare provider and aid-program records; peer-reviewed evidence of usability and technology acceptance; and extensive customer discovery defining the needs of families and providers. In an early staffing pilot, Olera received approximately 900 student applications, accepted 100 candidates, and placed 25 with local providers, with participating students and providers returning in a subsequent semester. These results support the CRP's next step: integrate, execute, measure, and commercialize the complete pathway.

Weaknesses in current approaches. Families do not lack individual resources; they lack a system accountable for carrying them across the full pathway to established care. Table 1 organizes current approaches around the same care-establishment pathway used throughout this application.

Pathway step	Where families still fall off	Olera's CRP approach
Assess Needs	Assessment may end as information or referral rather than an executable case	Persistent case begins with structured needs assessment
Identify Care	Fragmented inventories and handoffs leave families to reconcile options	Curated national resource infrastructure identifies services and providers
Fund Care	Eligibility information does not ensure applications, documentation, or approval	Aid identification is linked to execution and case tracking
Staff Care	Providers may have an opening but lack workers to serve the family	Caregiver Staffing adds a workforce channel when capacity blocks care
Execute Plan	Administrative tasks cross organizations; risk of "lost to follow up"	AI-supported workflows execute applications, documents, follow-up, and intake
Establish Care	Referral or eligibility is treated as success even when service never starts	Closed-loop confirmation records whether appropriate care actually begins

Track Outcomes	No longitudinal record connects need, pathway failure, and subsequent outcomes	Case-level outcomes infrastructure supports institutional evidence generation
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Table 1. Current approaches address portions of the pathway; the CRP integrates them into a closed-loop system oriented to confirmed care.

Commercial applications and innovation. The commercial opportunity is not another directory, referral marketplace, or staffing channel in isolation. It is an integrated infrastructure that carries a family across the care-establishment pathway and creates evidence about what happened at every step. AI-supported execution moves the product from recommending what a family should do toward completing and tracking the work required to establish care, and every executed case produces a structured longitudinal record of the pathway.

At scale, this longitudinal record could become a distinctive commercial and scientific asset: a county-level empirical map of where eldercare pathways succeed, where they fail, and what resolves those failures. The CRP tests and builds the infrastructure required to create this asset; it does not assume its value in advance (Figure 4, lower register).

Expected outcomes. (1) a verified CareNavigator shown in real-world use to execute a prespecified share of eligible administrative actions within bounded permission and human-escalation rules while tracking the pathway from care plan to established care; (2) real-world evidence on care establishment, failure points, operating cost, and longitudinal outcomes; (3) a repeatable Caregiver Staffing model that can both generate provider revenue and relieve workforce constraints to enable care establishment; and (4) a contracting-ready institutional evidence package and prespecified payer proof-of-concept design positioned for post-CRP validation, contracting, and private investment.

Commercial and non-commercial impact. Olera's commercial and public-health objectives reinforce one another: growth means more families can receive support before unmet needs progress to higher-cost crises.

Impact potential	Measured by	National priority served
Societal. Families establish appropriate care; available aid is converted into support; caregiving capacity is added where supply is short	Aid dollars secured; care established; time to care; staffed caregiver hours added	2022 National Strategy to Support Family Caregivers
Educational. Future health professionals gain supervised, real-world geriatric care experience while contributing to the direct-care workforce	Students recruited and placed; documented patient-care hours; retention	National direct-care workforce need
Scientific and public health. New visibility into where the care-establishment pathway fails and how patterns differ across communities	Case-level pathway records; failure points; county-level capacity and outcomes	Healthy aging, integrated care, long-term care access, implementation research

Table 2. Commercial growth produces measurable societal, educational, and scientific value.

3. Company

Origins and objectives. Olera, Inc. grew from a multidisciplinary effort at Texas A&M University to solve a problem families repeatedly described: eldercare was difficult to navigate, difficult to afford, and difficult to convert from information into actual support. PI Tokunbo (TJ) Falohun began working with Logan DuBose, MD, MBA, now Olera's Chief Research Officer and co-investigator, through the Texas A&M chapter of Sling Health in 2019. Their initial work focused on dementia caregiving; continued discovery revealed a broader need across eldercare and led to Olera's formation in 2020.

Olera's corporate objective is to make establishing eldercare navigable and broadly accessible for American families while building the commercial infrastructure that can sustain that access. **The commercialization strategy has evolved as evidence accumulated; the mission has not.** Core CareNavigator access remains free to families, recommendations remain neutral, and commercialization is concentrated where Olera creates incremental value for providers and institutional buyers.

Olera, Inc.	At a glance
Founded & structure	2020 · independent U.S. small business concern · C-corporation · founder-led Board

Commercial stage	Pre-scale commercial company · nationally deployed CareNavigator · early Caregiver Staffing validation · no significant annual sales
Leadership	TJ Falohun, PhD, CEO/PI · Logan DuBose, MD, MBA, Chief Research Officer/co-investigator
Team	Founder-led product, technical, and research leadership · existing full-time software engineer (~2 years) · second full-time software engineer added at CRP start · two full-time family/provider operations personnel (>3 years), including designated Market Operations Lead · part-time marketing · two part-time research assistants · Clemson human-subjects team
Federal R&D	≈\$5.7M NIA SBIR, 2021–2027 · Phase I/II Fast-Track + Phase IIB (1R44AG074116)
Commercialization development	NSF I-Corps customer discovery (200+ interviews) · Blackstone Techstars · Texas A&M
Senior advisors	Marcia Ory, PhD (>6 years advising Olera) · David Qu, MBA · Qiping Fan, MD, MS/Clemson University
Finance, compliance, regulatory	Founder-led finance and federal administration supported by ADC accounting/CPA and specialized legal/compliance counsel · HIPAA-aligned data practices · federal research, IRB, and data stewardship experience · referral-compliance review

Table 3. Olera at a glance.

Core competencies and operating continuity. Olera's capabilities now extend beyond the founders and reflect several years of accumulated operating experience. The company combines software and applied AI engineering, an expert-curated eldercare data infrastructure, human-centered aging research, digital distribution, family/provider operations, and commercialization research. Four peer-reviewed studies established usability and technology acceptance, and direct family/provider operations continue to expose the team to the practical barriers between a recommendation and established care.

Olera's progression from R&D to commercial scale. Early discovery defined the problem; Phase I/II built and evaluated the first-generation platform and national resource infrastructure. Phase IIB supported national deployment and deeper provider/workforce learning, and the CRP is designed to complete execution and outcomes capabilities, establish repeatable Caregiver Staffing economics, and build the evidence required for institutional commercialization. Successful CRP completion therefore changes the appropriate source of capital: subsequent scale is intended to be financed by commercial revenue and private investment rather than continued dependence on federal R&D support.

Workstream	CRP Y1	CRP Y2	CRP Y3
Product and technical leadership	Falohun	Falohun	Falohun
Engineering	Two full-time engineers and founder engineering	Maintenance and refinement	Commercial refinement
Research integration	DuBose	DuBose	DuBose
Human subjects	Clemson and Fan	Clemson and Fan	Analysis as needed
Family and provider operations	Two full-time existing staff	Eight free markets	Eight paid markets
Market operations lead	Training and playbook design	Runs validation activation	Executes new-market replication
Commercialization	Offer preparation	Willingness to pay and investor cultivation	Paid sales and financing
Independent validation	Technical and statistical, as specified	Statistical	CPA and economic

Table 4. Who carries each workstream in each award year, and how the staffing shifts as the work moves from engineering to validation to paid markets.

The lean internal team is complemented by senior expertise accumulated over years rather than assembled for a single application. Marcia Ory, PhD, has advised Olera for more than six years and brings decades of aging, caregiving, implementation, dissemination, and sustainability expertise, including 20 years at NIA. Qiping Fan, MD, MS, and Clemson University provide longstanding capabilities in epidemiology, mixed-methods evaluation, health-services research, and independent study execution. David Qu, MBA, brings approximately 30 years of healthcare-technology commercialization and executive experience, including scaling and exiting digital-health companies, together with relationships across healthcare and senior-care investment networks. TJ owns

provider commercialization because he is the CEO and must prove the motion. David pressure-tests the offer, financing strategy, and investor readiness. Aim 3 itself is intentionally the experiment that determines whether the company can execute commercialization beyond founder intuition.

As CRP milestones demonstrate repeatable provider sales, operating demand, and institutional engagement, Olera will internalize dedicated commercial, customer-success, operations, data/compliance, and finance capabilities when their workload and strategic importance justify full-time leadership.

SBIR/STTR commercialization history. Olera's SBIR history is one continuous arc: a single NIA project carried from concept to a validated national platform across a Phase I/II Fast-Track and a Phase IIB continuation (1R44AG074116). Table 4 answers the required history questions directly.

Required question	Answer
Company name changes in the past five years	None. The company has operated as Olera, Inc. throughout.
Subsidiary or spin-off status	Independent. No parent company.
Share of company revenue derived from SBIR/STTR funding, each of the past five years	FY2021 100% · FY2022 100% · FY2023 100% · FY2024 ~100% · FY2025 ~100% (one-off pilot fees, <1%)
Total SBIR/STTR Phase II awards from any federal agency	Two, on one project: the NIA Fast-Track Phase II and its Phase IIB continuation (1R44AG074116). No other agency awards.
Total revenues generated to date from commercialization of SBIR/STTR projects funded in the past five years	Limited and by design: multiple one-off paid pilots, no recurring revenue. Olera held monetization until the platform could support it without charging families or gating referrals. Provider willingness to pay also depends on concentrating families and providers in the same local markets, which research funding was not scoped to do.

Table 4. SBIR/STTR commercialization history.

4. Market, Customer, and Competition

Market segments and potential customers. Olera commercializes through two buyer classes created by the same care-establishment pathway. The near-term beachhead is care-delivery providers that lose revenue when caregiver vacancies prevent them from accepting or staffing new cases. The emerging institutional market is healthcare organizations that bear financial risk when unmet needs contribute to avoidable utilization, failed care transitions, or earlier institutional care.

Caregiver Staffing addresses an unusually persistent provider problem. The United States employed approximately 4.68 million home health and personal care aides in 2025, with roughly 760,500 openings projected each year from 2025 to 2035. Home-care benchmarking separately reported a median professional-caregiver turnover of 75% in 2024.^{5,6} The problem directly constrains growth: 63.3% of surveyed home-care providers reported turning down cases due to staffing shortages in 2023.⁷ Caregiver Staffing therefore addresses provider workforce demand. Non-medical home care is Olera's initial provider beachhead.

The institutional market is larger but evidence-gated. Prospective customers include Medicare Advantage plans, accountable care organizations (ACOs), health systems, Medicaid managed care and managed long-term services and supports organizations, and other entities exposed to the downstream costs of unmet need. In 2026, 35.2 million people are enrolled in Medicare Advantage, and 14.3 million Medicare beneficiaries receive care coordinated through accountable-care arrangements.^{8,9} CMS's active GUIDE Model further validates the purchasing logic as Medicare is already testing and paying for dementia care navigation, community-resource connection, and caregiver support, with the explicit aim of delaying nursing-home placement.¹⁰

Market opportunity and path to meaningful scale. Caregiver Staffing enters an existing, recurring recruitment market in which providers already spend to fill vacancies. The CRP determines Olera's repeatable pricing, conversion, retention, and unit economics. The CRP helps determine whether CareNavigator leads to more established eldercare and models if that outcome could create sufficient economic value for institutional contracting by reducing utilization of costly healthcare resources. Olera's geographic unit of commercialization is a county-centered local market which is defined as a service area anchored to one focal U.S. county or county equivalent, within which family demand, provider participation, workforce capacity, care-establishment outcomes, acquisition costs, and commercial performance are attributed and measured. With more than 3,100

such geographic units nationally, Olera can expand by replicating a validated local-market model without requiring dominant national market share.

Significant advantages and competitive position. The remaining relevant competitive question is not whether alternatives exist, but why Olera can create differentiated value as those alternatives evolve.

1. End-to-end execution. CareNavigator integrates assessment, care identification, funding, AI-agent execution, staffing when needed, and longitudinal outcomes rather than optimizing a single step.	2. New workforce supply and demand intelligence. Caregiver Staffing creates an easier pathway into direct care worker roles rather than redistributing workers already circulating among providers. CareNavigator reveals where family demand and provider capacity constraints collide locally.
3. Distribution and local data. Organic family demand, a national provider and benefits infrastructure, and county-level care-establishment and outcomes data compound as the platform is used.	4. AI interoperability. Olera can expose domain data and execution capabilities to search and general-purpose AI interfaces, allowing them to become entry points to CareNavigator rather than substitutes for its execution layer.

Caregiver Staffing's initial workforce wedge is deliberately narrow. Olera initially targets health-profession applicants and students for whom paid caregiving can also provide meaningful patient-care experience. The opportunity is nationally distributed and continuously replenished: in the most recent cycles, U.S. MD programs reported 54,699 applicants, and NursingCAS reported 75,078 applicants across 282 participating nursing schools.^{11,12} These two pipelines are not the full addressable workforce, as we plan to design a system that can eventually incorporate career changers, gig workers, and other well-positioned new labor pools into the direct care workforce. Traditional recruiting channels primarily compete for workers already searching for caregiver jobs. Recent recruitment benchmarking found Indeed generated 68% of applications to participating home-care agencies in Q1 2026, illustrating how concentrated conventional caregiver acquisition remains.¹³ Olera instead builds relationships with universities and applicant communities to introduce caregiving as a paid entry pathway into healthcare, while the licensed provider remains the employer responsible for interviewing, hiring, training, credentialing, supervision, and employment standards.

Current and emerging competition. For Caregiver Staffing, Olera competes across categories. Provider staffing alternatives include large job boards like Indeed, staffing agencies, caregiver-specific recruiting platforms, and emerging student-caregiver models such as CareYaya. For CareNavigator, alternatives include government and nonprofit resource directories, patient navigators, social workers and care managers, eldercare referral platforms like A Place for Mom or Caring.com, and increasingly general-purpose AI and search where families ask eldercare-related questions. The full customer segment and competitive environment is described in the Research Strategy Significance section.

Market and customer acceptance hurdles. The CRP is structured to measure the remaining commercial uncertainties rather than assume adoption.

Commercial hurdle	How the CRP retires it
Will providers pay?	Measure willingness to pay, conversion, retention, and unit economics for Caregiver Staffing across markets.
Can new workforce supply replicate?	Measure applicant acquisition, provider hiring, placement, retention, and market-to-market reproducibility.
Will institutional buyers have enough evidence to justify a payer proof-of-concept?	Generate verified care-establishment and pathway data, quantify operating economics, model downstream value with explicit causal boundaries, and produce a contracting-ready evidence package and post-CRP proof-of-concept specification.
Can AI execution be trusted and governed in real cases?	Verify bounded workflow classes, permission enforcement, administrative-error behavior, field-knowledge provenance/freshness, and user comprehension in Aim 1; then measure live authorization, agent-execution share, human escalation, material corrections, and safety events in Aim 2.
Can the model scale geographically?	Deploy across multiple counties and measure where performance, cost, workforce capacity, and care-establishment rates vary.

Table 5. Principal market-acceptance uncertainties and the CRP activity designed to retire each.

Strategic alliances, partnerships, and route to market. Olera enters the CRP with relationships on both sides of its beachhead. This includes university relationships that support workforce recruitment and working relationships with local and franchise-affiliated eldercare providers that can serve as early customers and

implementation sites. No FDA approval is required for the products proposed here, and Olera does not depend on a licensing agreement to commercialize them.

Provider sales begin where Olera can demonstrate an immediate staffing constraint and measurable hiring value. Institutional customer development begins with organizations whose populations and economics align with the outcomes the CRP is designed to measure. The detailed acquisition channels, sales process, production infrastructure, and post-CRP scaling plan are presented in the Production and Marketing Plan (Section 7).

5. Intellectual Property Protection

Protection strategy. Olera's defensibility does not depend on a single patent claim. The principal commercial moat is the accumulated combination of non-public execution logic, domain-specific data architecture, geographically conditioned operational knowledge, longitudinal care-establishment records, workforce infrastructure, and distribution channels required to reproduce the system. These assets receive different forms of protection according to how value is created. Source code and execution logic are restricted as trade secrets and protected by copyright. Proprietary database structure, derived variables, quality-control methods, field observations, and longitudinal datasets are access-controlled and governed through confidentiality and data-use obligations. Product names and branding receive trademark protection where appropriate, and CRP-generated inventions with separable patentable value will undergo counsel review before public disclosure. Olera will maintain an invention and disclosure review process throughout the award so that new workflow, data-modeling, execution, or workforce methods are evaluated for trade-secret versus patent protection before publication or other public disclosure.

Compounding barriers to replication. A new entrant would need to recreate Olera's national provider and benefits infrastructure, family distribution, Care Establishment Model, program- and provider-specific execution workflows, local field knowledge, workforce channels, provider relationships, and the longitudinal record of which pathways actually succeeded or failed. These assets compound through use. Each executed case adds timestamped operational observations, exposes missing or stale local knowledge, tests workflow behavior, contributes care-establishment outcomes, and improves the geographic model used by subsequent cases. Because this information largely does not exist on the public web and cannot be reconstructed retrospectively, the time required to accumulate comparable executed-case history becomes part of the moat.

6. Finance Plan

Capital required. Olera is requesting approximately \$4 million in CRP funding over three years to finance the later-stage R&D, real-world validation, and commercialization work required to cross the Valley of Death described in Section 1. The award is designed to move Olera from pre-scale commercialization to an investable commercial inflection point, not to assume that revenue from the limited CRP markets immediately replaces the full federal operating budget.

From CRP capital to commercial sustainability. Olera's financing strategy deliberately separates evidence generation from commercial scale. During the CRP, federal capital finances the engineering, real-world validation, and market experiments that private investors are not yet positioned to underwrite. Caregiver Staffing begins paid testing in eight new markets in Year 3, producing live conversion and unit-economics data. Task 3.2 simultaneously produces the evidence package required to secure a post-CRP institutional proof-of-concept. Independent third-party capital then finances expansion of the validated Staffing model and the payer validation step needed before institutional CareNavigator revenue is assumed. Commercial revenue progressively takes on a larger share of growth only after each revenue engine demonstrates its economics.

Fundraising Plan. Olera has deliberately financed its development primarily through non-dilutive SBIR support and has not yet completed a material institutional equity financing. The post-CRP raise would therefore represent an important transition in the company's capital structure, and the CRP is designed to generate the evidence required to make that transition on substantially stronger terms than are available today. Investor cultivation will begin by the end of Year 2, after technical verification and early real-world evidence are available. A formal financing process will begin during Year 3 as paid-market conversion, Staffing throughput, retention, unit economics, and the institutional evidence package mature. The objective is to enter post-CRP

commercialization with independent financing secured or actively closing rather than encounter a new Valley of Death at award completion.

Investor engagement. Commercialization advisor David Qu will advise Olera quarterly throughout the CRP, helping management define investment-readiness milestones, pressure-test the financing strategy and materials, and prepare for institutional fundraising. As milestones mature, he may support introductions and continued engagement with relevant senior-care and healthcare investors in his network, including Ziegler, Equitage Ventures, 7Wire Ventures, and Alumni Ventures, among others. His Letter of Support describes this role and commitment to helping Olera prepare for and pursue independent third-party financing as CRP milestones are achieved.

Financing readiness is milestone-based rather than calendar-based. The financing case will be built around evidence that is unavailable today: (1) technical verification of bounded agent execution and the Care Establishment Model; (2) verified care establishment across up to sixteen validation markets; (3) observed agent-execution share and human-escalation burden; (4) measured Staffing acquisition, placement, retention, and hires per market-month; (5) paid provider conversion, retention, CAC, cost to serve, and contribution margin across eight new commercial markets; (6) evidence that the market-entry playbook transfers beyond the founders; and (7) a contracting-ready institutional evidence package and payer proof-of-concept specification.

Post-CRP financing requirement. Olera expects independent third-party capital to finance the transition from CRP validation to commercial scale. With approximately \$1.4 million in expected operating requirements in Year 4 and roughly \$90,000 in modeled commercial revenue, and approximately \$1.85 million in Year 5 operating requirements against roughly \$400,000 in modeled revenue, Olera expects a substantial post-CRP financing requirement. The current target of approximately **\$3–5 million** will therefore be refined from the CRP's measured Staffing economics, demonstrated market-entry costs, institutional proof-of-concept requirements, and desired expansion pace rather than from optimistic revenue assumptions.

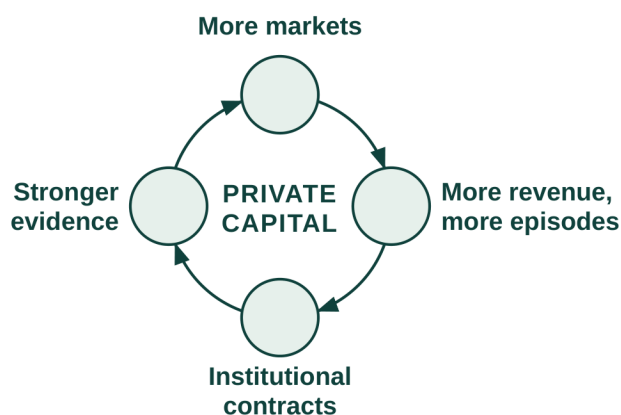


Figure 5. Post-CRP growth flywheel.

a county in which families can enter CareNavigator, providers can participate, workers can enter local provider labor pools through Caregiver Staffing, and completed care pathways can be measured.

Digital product and AI infrastructure. CareNavigator and Caregiver Staffing are developed by Olera's internal engineering team, including the founders and full-time engineering personnel. Because the products are software, there is no manufacturing, inventory, or physical-distribution dependency.

Market activation is centralized rather than locally staffed. The same software deployment, provider index, family intake, worker application system, communications infrastructure, and operations team serve every market. Geographic activation changes targeting, local data, provider/workforce concentration, and measurement rather than requiring a new office or dedicated local team. Aim 2 and Aim 3 therefore expand

Use of post-CRP capital. The raise is intentionally larger than the modeled operating deficit because its purpose is not merely to extend runway. Expanding Caregiver Staffing into additional county markets generates successful hires, provider revenue, and greater workforce capacity. Expanding CareNavigator generates more care-establishment episodes and a larger longitudinal evidence base from which institutional contracts can mature. Private capital therefore serves as the bridge from a CRP-validated model to progressively revenue-financed commercial scale (Figure 5).

7. Production and Marketing Plan

Production model. Olera produces and operates its software in-house, but the commercially meaningful unit of production is a functioning local market which is a

market count primarily through centralized digital acquisition and standardized operations, with human support allocated by case volume rather than county.

Data and workforce infrastructure. Olera's provider and benefits information is indexed, normalized, reviewed, and surfaced geographically so that local CareNavigator infrastructure can exist before every provider has actively joined the network. Caregiver Staffing operates through the same platform. Providers can seek workers for staffing needs arising anywhere in their business, while CareNavigator provides an additional signal when local workforce capacity is preventing a family from establishing care.

Market selection and activation. Olera will not enter counties indiscriminately. CRP markets are selected to support both rigorous testing and practical commercialization, using signals such as existing family demand, provider density and workforce need, recruitable workforce supply and nearby university or community-college infrastructure, existing local relationships, and the feasibility of concentrating enough participation to measure care-establishment and commercial outcomes. The CRP applies this process sequentially. Aim 2 activates eight free county-centered validation markets to measure care establishment, stakeholder acquisition, workforce throughput, geographic variation, and operating cost. Aim 3 then activates eight new paid markets, in two waves of four, to test whether the Aim 2 playbook transfers to previously untouched geographies and whether demonstrated provider value converts into real purchasing. The Aim 2 markets may continue operating and may later generate revenue, but they are excluded from the primary Aim 3 paid-conversion analysis because their prior free exposure could influence purchasing behavior.

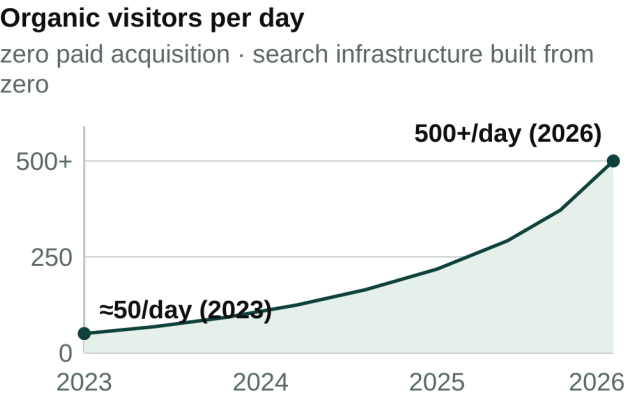
Customer and participant acquisition. A functioning Olera market requires concentrated participation from families, providers, and caregivers. Olera enters the CRP with established channels for each audience and will use the award to measure channel cost, yield, and reproducibility across markets (Table 6).

Audience	Established acquisition base	CRP market-concentration channels	Conversion/activation
Families	Organic search through provider pages, benefits resources, and guides; social communities	Geo-targeted search and social ads; community, faith, and aging-service organizations; clinics	Free CareNavigator account → assessment → active care-establishment pathway
Providers	National provider index; organic discovery; inbound profile claiming	Claim-your-profile email and calls; family inquiries; associations, conferences, franchise relationships	Listed → claimed → active → Staffing demonstrated → paid conversion
Caregiver workforce	University relationships and prior applicant pipeline	Career centers and job boards; student organizations; advisors and faculty; career fairs	Applicant → screening → provider interview/hire → staffed care
Institutional buyers	Advisor and industry relationships; emerging CRP evidence	Direct business development; advisor introductions; targeted outreach to MA plans, MCOs, ACOs, health systems	Evidence review → scoped proof-of-concept → utilization/economic validation → annual/population contract → broader deployment

Table 6. Distinct but coordinated acquisition channels concentrate the participants and customers each local market requires.

Figure 6. Organic traffic growth, 2023–2026.

Institutional buyer development. During the CRP, Olera will identify Medicare Advantage plans, Medicaid managed-care and managed-LTSS organizations, ACOs, health systems, and other prospective buyers whose populations and economics align with the measured care-establishment outcomes. Task 3.4 converts those results of Aims 2 and 3 into a buyer-facing evidence package and post-CRP proof-of-concept specification. The initial institutional sales motion is therefore evidence review → scoped proof-of-concept → utilization/economic validation → annual or population-based contract.



Distribution and route to market. Olera's distribution advantage is already visible in organic demand: traffic has grown from approximately 50 visitors per day in 2023 to more than 500 per day in 2026 without paid acquisition (Figure 6). Olera also intends to make CareNavigator's domain data and execution capabilities interoperable with search and general-purpose AI interfaces, allowing those systems to become additional entry points rather than requiring Olera to own every point of discovery.

8. Revenue Stream

Two revenue engines, sequenced by evidence. CareNavigator remains free to families, and basic family-provider connections remain free to providers. The near-term engine is Caregiver Staffing where providers pay when Olera helps them successfully hire workers. The emerging engine is institutional CareNavigator contracting to help reduce utilization of costly healthcare services for payer member populations.

Caregiver Staffing revenue follows successful hires. Successful hires x Realized revenue per successful hire x Active county markets is the equation used to estimate revenue potential. In Year 2, Olera deploys Staffing free across eight validation markets to establish applicant acquisition, provider hiring, placement, retention, market throughput, and geographic reproducibility before price is introduced. In Year 3, paid testing begins in eight new markets that did not receive free Aim 2 exposure. The working base-case price for this model is \$250 per successful hire, with \$150 and \$350 as sensitivity bounds. This is an Aim 3 pricing hypothesis, not an asserted market price. It is economically plausible relative to the burden providers already bear in hiring care workers (Table 7).

Model input	Base projection	Range or benchmark	Basis
Successful hires per active market per month	2-5	1 to 5 sensitivity	Illustrative pre-CRP planning assumption; Aim 2 measures actual market throughput
Price per successful hire	\$250	\$150 to \$350 sensitivity	Aim 3 pricing hypothesis
Home-care turnover		About 75%	Published benchmark ⁶
Recruiting and training burden		Up to about \$2,700 per replacement	Published benchmark ¹⁴
First institutional proof of concept work	Post-CRP Year 4		Contracting/validation milestone; revenue not assumed in base case
First annual institutional relationship	Post-CRP Year 5		Evidence-gated planning assumption
Institutional relationships	About 1 by Year 5; 3-10+ longer-term sensitivity		Planning assumption after successful proof-of-concept
Annual value per validated relationship	About \$250K	Negotiated after proof-of-concept; sensitivity to be developed from buyer evidence	Planning hypothesis, not a benchmark

Table 7. The projection separates published benchmarks from Olera assumptions and CRP hypotheses.

The pre-CRP financial model therefore uses an illustrative and conservative base case of two successful hires per active market per month. At the \$250 per hire charged to the provider as our Aim 3 pricing hypothesis, the base case equals \$6,000 annualized Staffing revenue per active market and \$48,000 across eight paid markets. Aim 2 will replace the throughput assumption with observed hires per market-month before Aim 3 pricing and paid conversion are interpreted.

The longer-range scenarios illustrate geographic replication rather than forecast a mature national throughput rate. At the two-hire-per-market-month base case, 100 active markets would produce approximately 2,400 successful hires and \$0.6 million in annual Staffing revenue; 250 markets, approximately 6,000 hires and \$1.5 million; and 500 markets, approximately 12,000 hires and \$3.0 million. We estimate a total addressable market of 1233 mid-sized counties, representing ~66% of the 3000+ counties in the US.

Institutional CareNavigator: contracts follow outcomes evidence. The institutional buyer class is illustrative of the pre-CRP base case. CRP years 1 to 3 test whether recognized needs reach established care, why pathways fail when they do not, and what happens longitudinally afterward. This matters economically

because unmet home- and community-based service needs have been associated with substantially greater acute-care utilization (Section 1).⁸ There is already precedent for organizations responsible for health outcomes to pay for care-management and coordination infrastructure: CMS's GUIDE Model uses per-patient-per-month dementia care-management payments for coordination and caregiver support, and in 2026 the Medicare Shared Savings Program includes 511 ACOs serving 12.6 million Traditional Medicare beneficiaries.^{9,10} These sources establish the buyer class and purchasing logic but they do not establish Olera's future price. Accordingly, the base case assumes no institutional revenue during the CRP or the first post-CRP proof-of-concept year in Year 4. Institutional revenue enters the base case only after proof of concept in Year 5, beginning with one approximately \$250,000 annual relationship.

	CRP Y1	CRP Y2	CRP Y3	Post-CRP Y4	Post-CRP Y5–Y10 potential
Commercial stage	Build	Validate free	Monetize	Expand	Scale to national replication
Validation markets	0	8, free	~8 continuing**	–	–
New paid replication markets	0	0	8 new paid	~15 total paid	~25 to 500 total paid
Staffing revenue	\$0	\$0	~\$24K*	~\$90K	~\$0.15M–\$3M/yr
Institutional proof-of-concept / annual relationships	0	0	0	~1 proof of concept	~1 annual relationship: ~3 to 10+ longer-term potential
Institutional revenue	\$0	\$0	\$0	~\$0 base case***	~\$250K base case; ~\$0.75M –\$2.5M+/yr longer-term
Total commercial revenue	\$0	\$0	~\$24K	~\$90K	~\$400K base case, expanding with measured Staffing throughput and validated institutional contracts

Table 8. Five-year evidence-gated commercialization base case.

Staffing and capital as revenue grows. During the CRP, engineering, research, and market-validation personnel remain central. As paid Staffing expands, Olera adds centralized market operations, worker acquisition, provider success, and sales capacity rather than recreating a full team in every county. Engineering grows more slowly because the platform, portals, and workflows are designed for self-service and automation.

9. Project Management Plan

Team, workstream ownership, and governance. Falohun retains final go/no-go authority and owns product, engineering, agent-execution QA, and provider commercialization. DuBose owns research integration, cross-aim operations, the milestone calendar, and safety-event coordination. Fan/Clemson owns human-subjects protocol execution and mixed-method evaluation, with Ory providing senior scientific guidance. An existing full-time family/provider operations team member will serve as Market Operations Lead for Aim 2 activation and the non-founder Aim 3 replication test. Olera's existing and newly added full-time engineers execute the technical work under Falohun.

Commercialization timeline and gates. The detailed experimental timeline and thresholds are provided in the Research Strategy. The management timeline below shows how those activities produce successive commercialization decisions.

Period	Commercialization milestone	Decision or financing gate
Year 1 (CRP)Build	Technical and measurement infrastructure operational; Staffing workflow ready for real-world deployment.	Confirm readiness for real-world market validation.
Year 2 (CRP)Validate free	Enroll approximately 440 index family episodes across eight validation markets; retain ≥400 analyzable episodes; quantify verified care establishment, agent-execution	Advance to paid testing only if human-protection criteria remain satisfied, the care-establishment gate is met, the verified automation envelope performs predominantly through agent execution rather than

	share, family authorization, human escalation, stakeholder acquisition, workforce throughput, and operating cost.	navigator execution, and the operating evidence supports a credible paid test. Use Aim 2 results to finalize the Caregiver Staffing product configuration, automation envelope, acquisition playbook, Staffing throughput assumptions, and paid offer that advance into eight new Aim 3 markets.
Year 3 (CRP)Monetize	Paid Staffing operates in the Aim 3 markets and produces live conversion, throughput, pricing, retention, and unit-economics records; at the illustrative pre-CRP base case, approximately \$24K would be recognized over six paid-month equivalents and the annualized exit run rate would be approximately \$48K.	Commercial progression is determined by measured conversion, throughput, retention, contribution margin, and repeatability rather than achievement of the illustrative revenue projection. Determine whether new-market activation, paid conversion, retention, and unit economics are sufficiently repeatable to justify post-CRP expansion beyond the eight paid CRP markets.
Year 4 (post-CRP)Expand	Expand paid Staffing according to measured CRP economics and secure/execute the first payer or risk-bearing institutional proof-of-concept. The illustrative base case assumes approximately \$90K Staffing revenue and no institutional revenue.	Institutional annual contracting advances only if the proof-of-concept produces sufficiently credible utilization, economic, and implementation evidence; otherwise refine the target population, intervention, or study design before broader institutional commercialization.
Year 5 (post-CRP)Scale	Expand Staffing toward approximately 25 paid markets and, if the Year 4 institutional proof-of-concept supports the value proposition, convert at least one institutional relationship into an annual commercial contract. The illustrative base case is approximately \$400K combined revenue.	Scale each revenue engine according to its independently measured economics rather than requiring institutional contracting to subsidize an unproven Staffing model or vice versa.

Table 9. Five-year commercialization management timeline. Revenue values are planning projections rather than CRP success criteria.

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